



Care of the Person Who May Be Suicidal (Acute) Policy

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Policy Writing Guide Summary](#) for further information about Scope of Practice.

Background

The purpose of this policy is to guide staff within the RKPG to support the provision of evidence-informed clinical care for people at risk of suicide, aimed at maximising their safety and supporting their recovery **wherever they may present within the hospital group**, as per the [Department of Health \(DoH\) Safety Planning for Mental Health Consumers Policy](#).

Values underpinning care (see also Appendix B)

- The RKPG recognises that supporting people who may be suicidal requires clinicians to orientate their interventions to support the patient's recovery, and not solely focus on maintaining their safety.
- Supporting a person who may be suicidal requires a collaborative approach involving the patient and their carers/family/support networks.
- The RKPG recognises that the factors influencing a person who may feel suicidal are often complex and multi-layered. They are often non-clinical in nature and reflect challenging social and cultural dynamics. As such the RKPG values the patient's account of their feelings and intent, and hold patient's individual experience at the centre of developing the patient's care and management plan.
- A capacity and risk assessment must be considered and documented when developing an appropriate response in all cases, particularly if the Senior Clinician is considering detaining the patient for their own safety.
- The age and development of the individual must be considered in developing an appropriate response in all cases.
- People who may be suicidal can present at any patient contact point within RKPG Services and as such this framework applies across all directorates including community services.

RESPONSIBILITY				NATIONAL STANDARDS
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- Recognising that Aboriginal and Torres Strait Islander populations have higher rates of suicide than non-indigenous populations, this RKPG policy recommends that clinicians consider involving an Aboriginal Liaison Officer (ALO) in relevant situations when considering collaborative strategies with the person. There ALOs employed at Rockingham General Hospital (RGH) Monday to Friday and Aboriginal Mental Health Liaison Officers employed in the community mental health service.

Aboriginal Liaison contact numbers

Site	Contact number
RGH	9599 4520
Rockingham CMHT	9528 0600
Peel CMHT	9531 8080

Assessment of people who may be suicidal

- The assessment and process applies to any person who presents to any part of RKPG following:
 - * An intentional act that was undertaken to end their life.
 - * An act that a reliable third party (e.g. close friend, family, Police Officer) believes was intended to end their life.
 - * An act that the person states was undertaken when they wanted to end their life.
 - * A verbalised intent to end their life.
 - * **Being under the influence of alcohol or other drug does not exclude them from this process.**
- All people who present as or become suicidal require prompt assessment of their psychiatric, psychosocial and other risk factors that may influence the likelihood of them attempting or completing suicide.
- Once identified and assessed by a Senior Clinician (including immediate capacity and risk considerations), the person should be supported whilst a psychiatric referral is undertaken. Refer to section 'Referral for psychiatric assessment' page 4.
- As there may be a delay between identifying the person as at risk and accessing appropriate mental health support, all staff should be familiar with the principles of supporting a person who may be suicidal as described below.

Suicide Intervention Triangle - options for intervention when someone voices intent to suicide or immediately after someone has attempted suicide

- When a person voices intent to suicide or has attempted suicide immediate action is required to maintain their safety, including notifying the Medical Officer (MO)/Nurse Unit Manager (NUM) and Medical Emergency Team (MET) as required.

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The suicide intervention triangle provides a simple guide to assist in taking immediate action to decrease risk.

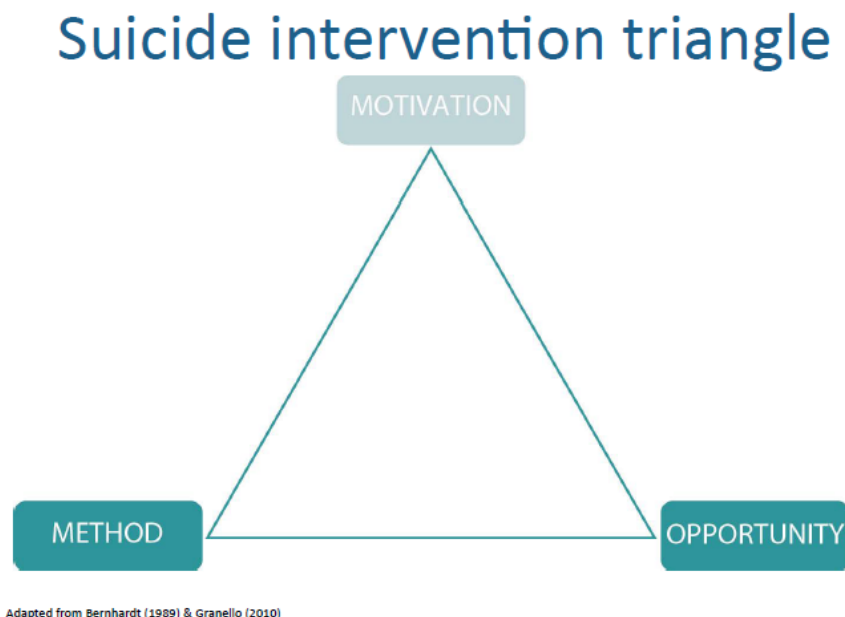
- The **two** immediate responses to a suicide crisis include:

1. Method

- Reduce access to means:
 - * Check the environment and individual and remove:
 - Dangerous items.
 - Ligatures (i.e. belts/shoelaces).
 - Medication.
 - Sharps etc.

2. Opportunity

- Reduce the opportunity for the person to suicide:
 - * Supervision 24/7 (if appropriate continuous supervision).
 - * Staff 1:1.
 - * Family/friend/carer involvement.
 - * Hospitalisation.



- Motivation is faded in the suicide intervention triangle (above). This is because in a suicide crisis it's important to address safety. Motivation for suicide can be addressed when the crisis has passed.

Assessing suicidality

- **There is no evidence to suggest asking about suicide will increase risk.**
- If the person's presentation meets the definition at the beginning of the policy then it is likely that the person will require a referral for more specialist assessment. This

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should be discussed with the senior clinician on duty and/or a treating doctor/Consultant.

- If, when talking to the person the conversation suggests that the person does not want to kill themselves then a referral for further assessment may still be indicated.
- It is suggested that in this instance staff express to the person that they remain concerned about the person's safety and consider increasing the level of engagement and observation of that person until their care can be reviewed.

Assessing acute on chronic suicidal ideation

- Some patients may present with enduring and long term thoughts of suicide.
- There is an evidenced heightened baseline risk of suicide associated with this group of patients presenting with recurrent or persistent suicidal ideation.
- Whilst some risk factors are long term or stable, the dynamic and fluctuating nature of risk in response to stressors, will impact on the intensity of the individual's thoughts to suicide.
- Initial assessment should follow the principles outlined previously in this policy.
- It is important to further evaluate this risk and develop a supportive plan of care as there may be certain identifiable circumstances that would increase this level of risk whilst in the hospital setting.
- If a person voices suicidal ideation of any nature, a capacity and risk assessment needs to be considered for immediate care delivery and a referral for a psychiatric assessment should occur.
- A comprehensive mental health assessment and care planning should identify chronic thoughts and plans for suicide and highlight potential areas/triggers which could increase risk and prompt an increased level of assessment, support, visual observations and further assessment of risk for containment.
- Documentation of the mental health assessment will follow the process as described in the [DoH State-wide Standardised Clinical Documentation for Mental Health Services](#).
- Once referred for a psychiatric assessment documentation of risk should be completed via mental health staff, using the (SMHMR902 form), refer Appendix B.
- A PSOLIS alert must be entered highlighting the chronic risk of suicide and identifying triggers/stressors which would acutely increase risk.
- Formulation of a crisis care or management plan must outline chronic risk and specific details of factors that would increase risk for the individual patient. Principles should follow the [DoH Safety Planning for Mental Health Consumers Policy](#).

Promoting safety

In addition to the initial management suggestions in the suicide intervention triangle above, staff could consider (also see Appendix A):

- Discussing with the patient what would help them feel safer.
- Involving a member of the patient's family or friends to support the patient.
- Appropriate medication.
- Distraction techniques.

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- Move patient to a visible area if concerned.
- Discuss/escalate to NUM/ Duty Nurse Manager (DNM) if 1:1 special required.

Referral for psychiatric assessment

- Within RGH this may mean assessment by the Psychiatric Liaison Team based in the ED or an e-referral to the Consultation Liaison Team (Pager 717), or out of hours On-Call MO DECT 94994.
- At MDH or Mandurah Community Health Centre this may require a referral to Peel Assessment and Treatment Team (9531 8080).
- The referral can be made by phone or taking the person to Peel ED (9531 8000).
- Within Peel Health Campus Emergency Department this will usually mean assessment by the Psychiatric Liaison Nurse (PLN) between 0700-2300hrs. Outside of these hours a risk assessment will be conducted by duty medical staff, with consideration given to keeping at-risk patients for a formal review by the PLN in the morning. All risk assessments should involve a review of the clinical information available in PSOLIS.

Patients wishing to leave prior to formal psychiatric assessment

- Patients with a clear identified risk of suicide who are wishing to leave the healthcare environment should, according to the principle of least restrictive means necessary, be initially encouraged to stay, but failing this it may be appropriate to detain them under either the [Mental Health Act 2014](#) or “Doctrine of Necessity” provisions (see Appendix B regarding “Doctrine of Necessity”).
- In instances where the clinician caring for the patient believes that such patient will be at imminent risk to their own safety if allowed to leave the facility the clinician should:
 - * At no time endanger themselves or put themselves at unnecessary risk.
 - * Provide a clear explanation of your immediate concern for their safety and ask them to stay until seen by a doctor or the mental health team. Offer support information (see Appendix A).
 - * Offer to make a warm drink and provide support.
 - * Ask if there is a particular reason they want to leave.
 - * Seek to find a solution to issues driving their desire to leave (if appropriate).
 - * Make a note of clothing and distinct features.
 - * If still wanting to leave, inform them that due to your concern you are required to inform their next of kin (NOK) and Police if they leave.
 - * If you feel you have lost control of the situation and cannot contain risk activate a Code Black and await support.
 - * When Security and the DNM arrive inform Security you are detaining the individual under Doctrine of Necessity until the patient has been reviewed by the mental health team or senior medical staff member.
 - * If the patient has already exited the area:
 - Request security performs a check of hospital grounds.
 - Inform NOK/carer/family of situation.

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- Inform treating Consultant or on call Consultant.
- Inform NUM or DNM.
- Contact Police and complete the WAPOL form as per [RKPG Missing or Suspected Missing Patients \(Acute\) Policy and Procedure](#).
- Document the incident in progress notes and outline all steps taken to encourage the patient to remain onsite and actions taken once the patient exited area of care.
- Complete DATIX notification.
- It is vital that some form of risk assessment/formulation is clearly documented in the clinical notes before a patient with suicidal ideation is discharged home. In addition, every effort should be made to ensure a safe discharge plan. Examples of this may involve (if consent previously obtained) the patient's family/supports, their General Practitioner (GP), Mental Health Assessment and Treatment Team (ATT) follow-up, and/or appropriate medications, and it should be clearly documented in the patient's medical record.
- Consideration should be given to using the guidance contained in the [RKPG Missing or Suspected Missing Patients \(Acute\) Policy and Procedure](#) supporting the consideration of requesting a Police welfare check for people who are assessed as being at risk of harm but not detained under the [Mental Health Act 2014](#).
- A Medical Officer (MO) or Authorised Mental Health Practitioner may consider the use of the [Mental Health Act 2014](#) as a means of containing the person and keeping them safe.

Mental health assessment

- Once a person is deemed at risk, they will require a comprehensive mental health assessment via mental health staff, using the (SMHMR902 form), refer Appendix B.
- The mental health assessment will follow the process as described in the [DoH State-wide Standardised Clinical Documentation for \(SSCD\) Mental Health Services](#).
- Also refer to the [SMHS Triage Assessment of Referrals in Community Mental Health](#)
- Following assessment, where possible, a collaborative plan of care involving the person and an identified friend or family member will be created. This will be documented appropriately.
- As a minimum it is suggested that the information in Appendix A is completed and given to the person (with a copy filed).

Postvention

Postvention is the term used regarding the care provided following a completed suicide. The effects of suicide on the bereaved can be significant and there is documented evidence that the suicide can trigger others to consider this act for themselves.

RGH Postvention

- Patients may be admitted to RGH following an attempt to suicide and then subsequently die. In all cases where this occurs the coroner is informed ([SMHS Deaths Reportable to the Coroner Policy](#)).

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- The Coroner's counselling service will contact the identified bereaved and offer postvention counselling. Typically this takes 3 weeks for contact to be made.

RKPG Postvention

- (For people who may die following a suicide attempt who resides in the RKPG catchment area).
- Staff supporting the bereaved will ask if the bereaved would consent to share their contact details with Peel or Rockingham Community Mental Health Teams so that timely appropriate postvention can be provided.
- This is the access point for the PaRK Community Response Group who can make contact with the bereaved, assess support needs and coordinate appropriate community responses. This may include speeding up access to specialist suicide bereavement counsellors, communicating with friends/family, identifying groups at risk as a result of the suicide and sign posting support services for people who may be triggered by the news of the death.
- In this situation staff can contact Peel ATT on 9531 8080 and pass on the relevant details for a postvention response.

Suicide completed within the hospital

- Refer to [RKPG Resuscitation: Code Blue Medical Emergency \(Acute\) Policy](#).
- The Police must be called if a patient within the hospital completes suicide whilst attending for a service (outpatient). The Police investigation, acting on behalf of the coroner, will take primacy over all other hospital processes.
- Post vention support should be given as described above to the bereaved and any staff or visitors who may have been affected.
- First responders are particularly at risk of psychological trauma and may well require ongoing support.
- The incident will be reported in DATIX and investigated as a SAC 1 clinical incident.

Compliance monitoring

- All cases where it is suspected that a person admitted to care within the RKPG has completed suicide will be reviewed and investigated as a Severity Assessment Code (SAC) 1 event, a record of which is maintained by Education, Quality Safety and Risk Unit.
- All cases where it is suspected that a person has attempted to end their life whilst on the premises of RKPG or in the care of a community service will be reviewed through the SAC process, a record of which is maintained by Education, Quality Safety and Risk Unit.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[DoH Safety Planning for Mental Health Consumers Policy](#)

[DoH State-wide Standardised Clinical Documentation for Mental Health Services](#)

[Mental Health Act 2014](#)

[RKPG Missing or Suspected Missing Patients](#)

[RKPG Resuscitation: Code Blue Medical Emergency \(Acute\) Policy](#)

[RKPG Resource Management for Patients Requiring Increased Observations Policy and Procedure](#)

[Saxinstitute Management of suicidal behaviour – evidence for models of care: a rapid review.](#)

[SMHS Triage Assessment of Referrals in Community Mental Health](#)

[SMHS Deaths Reportable to the coroner](#)

National Standards



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Appendix A: **Mental Health Contact Information and Safety Plan**

Follow Up

- If you (or the person you care for) have been asked to follow up with a GP it is important that you arrange an appointment as soon as possible.
- If you (or the person you are caring for) are in crisis or feel at risk, help is available from:

Child and Adolescent Mental Health (CAHMS) (0-18yrs)

Rockingham Tel (08) 95280555
Peel: Tel (08) 95318080

Adult Community Mental Health Services (these services have an Aboriginal Mental Health Liaison Officer)

Adult Mental Health Services (18-65yrs)

Rockingham Tel (08) 95280600
Peel Tel (08) 95318080

Older Adult Mental Health (65+):

Rockingham & Peel Tel (08) 65574900

Emergency numbers

Mental Health Emergency Response Line (MHERL)

Has Senior Mental Health clinicians available who, if required, can facilitate urgent assistance.

MHERL is available 24 hours a day, regardless of your location or age.

Perth Metro (local call) Tel 1300 555 788

Peel (free call) Tel 1800 676 822

Lifeline: You (or the person you care for) can call lifeline 24-hours a day. You can speak to a trained crisis support worker.

Tel: 13 11 14 or visit www.lifeline.org.au

Beyond Blue

Tel 1300224636 or visit <https://www.beyondblue.org.au/>

Medication

- You (or the person you care for) can further discuss medication with:
 - * Your local mental health clinic
 - * Your GP
 - * Local pharmacist
 - * Medicinewise – Tel: 1300 633 424 or visit www.nps.org.au
- **Alcohol and Other Drugs**
 - * If there are concerns related to alcohol and other substances please contact
 - * South Metro Community Alcohol and Drug Services:
 - * Rockingham Tel (08) 9550 9200 or
 - * Mandurah Tel (08) 9581 4010 or
 - * Visit www.palmerston.org.au

Community Mental Health

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- If you (or the person you care for) have been referred to our Community Mental Health Service you will be contacted by a Mental Health clinician to discuss your care.
- If you have not been contacted in 2- 3 days please call the appropriate clinic.

(Example below of a safety plan that can be written in the patient's medical record)

My Personalised Safety Plan

Date completed

My own abilities & strengths

.....
.....
.....

My Personal Support Networks & Contact Details

.....
.....
.....

My Other Support Networks

.....
.....
.....

My Follow-up Care

.....
.....
.....

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Appendix B: **Values and explanations underpinning this policy.**

- **Doctrine of Necessity** – Doctrine of Necessity is a common law doctrine, meaning it has been developed by the courts on a case by-case basis. The Doctrine of Necessity can be used to formulate a defence or rationale for the use of restrictive practices. This is relevant for the period of time prior to psychiatric assessment where clinicians have concern for the safety of the patient but the patient may be trying to leave the clinical setting.

The defence of necessity requires the following:

1. The act (restrictive practices) was done to avoid immediate risk/harm to the person or others.
2. The person(s) carrying out the act did so from an honest and reasonable belief that the individual was placing themselves or others in a situation of immediate risk/harm.
3. The act was proportionate to the harm about to occur.

Within a medical context if the patient can demonstrate capacity but the Senior Clinician is certain of imminent harm occurring then measures can be authorised and taken to ensure the patient remains in hospital for psychiatric evaluation or ongoing care.

The elements of the Doctrine of Necessity must be documented in the patients integrated notes by the Senior Clinician including the following:

- * Capacity assessment
- * Risk assessment (risk to self, risk to others)
- * Authorised proportionate act (detention, physical restraint etc.)
- * Follow up plan for ongoing delivery of care/treatment
- **A Psychiatric assessment** evaluates symptoms, their mental state and any past mental health history to determine if an untreated, relapsed or previously undiagnosed psychiatric disorder may be the cause of the clinical presentation.
- Recognising that many people present as suicidal without an identifiable psychiatric disorder, it is essential to assess **psychosocial factors**. These can include but not be restricted to stressors, significant recent life changes events including recent loss, identification with a person who may have completed/attempted suicide, use of alcohol and other drugs which may increase impulsive behaviour. It is important to assess protective psychosocial factors as well such as strong social supports, problem solving skills and the ability/willingness to seek help.
- **Assessment of risk** determines the severity of self-harm, suicidal thoughts or behaviour including identifying any specific plans for suicide, access to means, potential lethality of the chosen method, persistence of ideation, what precautions against discovery were planned, impulsivity and distorted thinking, and details of any

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previous suicide attempts. Risk assessment checklists or tools have low reliability for predictive purpose and, while they may aid clinical decision-making, should not be used in isolation to assess risk and determine clinical care.

- This information should be gathered in collaboration with the patient and any carers/family/friends and recorded the clinical record, and where appropriate in the Statewide Standardised Clinical Documentation (SSCD) (SMHMR902) form. Although there are circumstances where a clinician is working alone, most assessments and decisions regarding treatment and safety should be made by a multidisciplinary team in collaboration with the patient and their family and personal support person.

Responding to a person who may be suicidal and developing a safety plan

- Once a person is deemed at risk, they will require a comprehensive mental health assessment using the (SMHMR902 form). This includes those who present to health services with self-harm, suicidal ideas, in crisis and acute emotional distress. In settings where such an assessment is not available a referral to Mental Health (intra hospital or community) is advised. ED Mental Health Liaison/Consultation Liaison or Community Mental Health Triage can guide these. Non mental health care providers including general trained clinicians and carers can be valuable partners in recovery and can support the Safety/Care Plan.
- The development of a Safety Plan in partnership with the client needs to bridge immediate risk and ongoing recovery. It should therefore 'map' a path towards recovery including accessing follow-up; where appropriate, therapy; help with decision making and choices. This can be incorporated as part of the treatment, support and discharge plan (SSCD SMHMR907) outlined in the [Mental Health Act 2014](#).
- Some clients may have persistent and/or recurring risk which can be particularly challenging to all involved with the person. Carers and providers may seek guidance/advice/support from mental health services.

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